



✓ No new health zones have been affected in the last 24 hours.

ACCUMULATIVE
CAS
7 541

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CAS

7 541



ACCUMULATIVE
DEATH
CONFIRMED
3 639



LETHALITY
48,3%



PATIENTS IN ISOLATION/CTE



ACCUMULATIVE
HEALED
 1 823



TRACKING RATES

CONTACTS
(Of the Day)

87.2%

Uele, Ituri, North Kivu,
South Kivu, Tshopo, Lower Uele and
Sud Ubanqi



62 out of 167
health zones affected
(37,1%)

hours. In total, **62 out of 167 health zones (37.1%)** are affected in **7 provinces**. Ituri has 28 out of 36 health zones affected, followed by North Kivu (16/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34), Bas-Uélé (3/11) and South Ubangi (1/16).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 17, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	37	5829	2 661	45,7%	28/36 (77,8 %)
North Kivu	20	1370	837	61,1%	16/34 (47,1 %)
Haut-Uélé	4	296	123	41,6%	6/13 (46,2 %)
Boss	5	35	12	34,3%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	7	4	57,1%	3/11 (27,3%)
Sud Ubangi	0	1	1	100,0%	1/16 (6,3%)
Total	66	7 541	3 639	48,3%	62/167 (37,1 %)

Table 2. Distribution of confirmed cases and deaths by province and zone as of September 17, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5829	2661	45,7%	37	7	12	19
Give	11	1	9,1%				0
Underwood	8	6	75,0%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	186	39	21,0%	3	0		0
have	2	2	100,0%				0
Bunia	1635	512	31,3%	15	1		1
Ladies	40	9	22,5%				0
Drodre	15	7	46,7%				0
Fireworks	80	30	37,5%				0
Gethy	7	3	42,9%				0
Flounder	2	0	0,0%				0
Kilo	34	12	35,3%				0
Team	141	88	62,4%	3	0		0
Liter	269	133	49,4%	2	1		1
Logo	13	5	38,5%				0
Fight	31	8	25,8%				0
distributed	4	1	25,0%				0
Washing	21	9	42,9%				0
Darkness	59	33	55,9%				0
Mangala	326	146	44,8%				0
Mongbwali	676	312	46,2%	4	4		4
Nanny	253	132	52,2%				0
Nizi	748	274	36,6%	5	1		1
Legumes	125	36	28,8%				0
Jungle	12	4	33,3%	1	0		0

Rwampara	1036	350	33,8%	4	0		0
Tchomia	75	25	33,3%				0
To be ventilated*	THAT	477	THAT			12	12
North Kivu	1370	837	61,1%	20	7	2	9
Me	263	186	70,7%	8	3		3
Farm	18	7	38,9%				0
Butembo	246	160	65,0%	2	2	1	3
Ten	1	0	0,0%				0
Kalunguta	40	13	32,5%	4	0	1	1
Cut	538	340	63,2%	2	1		1
Source	1	0	0,0%				0
Kyondo	32	20	62,5%				0
Lubero	15	7	46,7%				0
Be careful.	23	10	43,5%	1	1		1
Manguredjipa	10	3	30,0%				0
Masereka	24	9	37,5%				0
The museums	119	64	53,8%	1	0		0
Mutwanga	8	2	25,0%	2	0		0
Oicha	12	10	83,3%				0
Vuh	20	6	30,0%				0
Haut-Uélé	296	123	41,6%	4	1	4	5
Kill Mangbetu	35	15	42,9%	1	0		0
Gombari	3	1	33,3%				0
Mathematics	98	42	42,9%				0
Lining	63	30	47,6%	0	0	4	4
Beans	2	1	50,0%				0
Ordinary	95	34	35,8%	3	1	0	1
Boss	35	12	34,3%	5	1	0	1
Bafwasende	7	1	14,3%				0
Kabondo	4	1	25,0%				0
Lubunga	1	0	0,0%				0
Maquis-Kisangani	17	7	41,2%	5	1	0	1
Mangobo	4	2	50,0%				0
Boss	1	0	0,0%				0
Wanie-Rukula	1	1	100,0%				0
South Kivu	3	1	33,3%	0	0	0	0
Mili-Murhesa	3	1	33,3%				0
Bas-Uélé	7	4	57,1%	0	0	0	0
Blind	1	1	100,0%				0
Viadana	3	1	33,3%				0
Walk	3	2	66,7%				0
Sud Ubangi	1	1	100,0%	0	0	0	0
Hair	1	1	100,0%				0
Total	7541	3639	48,3%	66	16	18	34

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

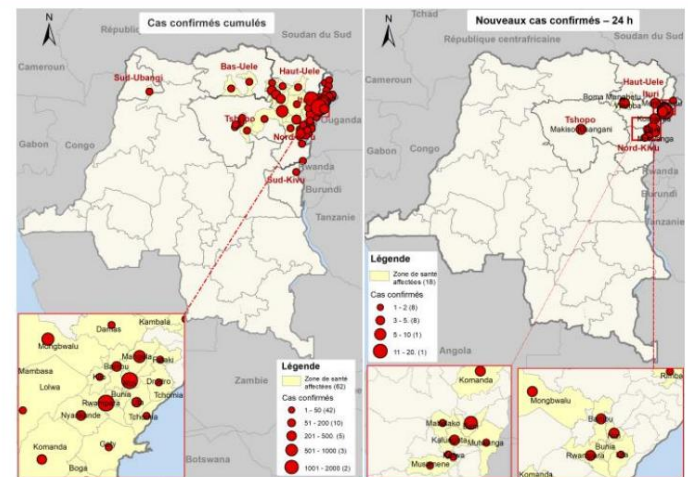
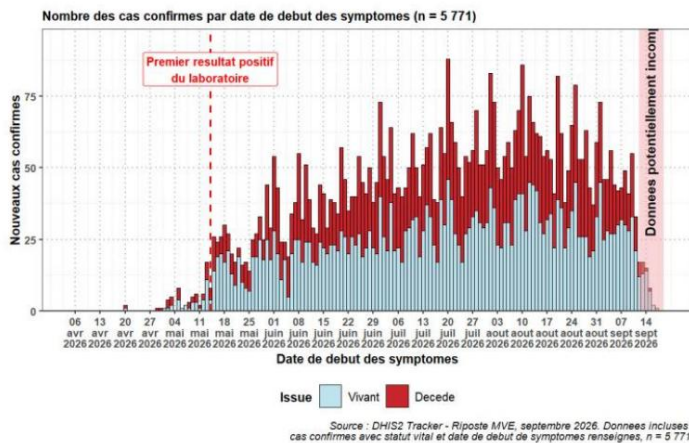
NA : Not Applicable.

Table 3. Status of alerts notified by province for September 17, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CTE/CT (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	74	4	78	20	3	43	1	23	13
Bas-Uélé	97	3	100	12	3	84	0	15	0
Ituri	1 082	152	1234	951	107	131	9	1058	90
North Kivu	1 169	59	1228	159	59	747	0	218	94
South Kivu	9	0	9	2	0	6	0	2	0
Sud Ubangi	15	0	15	3	0	12	0	3	0
Boss	151	4	155	8	2	145	0	10	4
Total General	2 597	222	2 819	1 155	174	1 168	10	1 329	201

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.1.2. Main Actions

On September 17, 2026, 2,819 alerts were reported, of which 2,507 (88.9%) were verified. Among these, 1,329 alerts were validated as suspected cases.

All of these suspected cases (100%) were investigated, and 201 were transferred to CTE/CT (Table 3).

Of the 31,902 contacts currently being monitored, 27,842 were seen in the last 24 hours, representing a follow-up rate of 87.2%, stable compared to the previous day (87.6%). This rate was 91.7% (14,377/15,682) in Ituri; 88.2% (186/211) in Bas-Uélé; 83.3% (11,431/13,717) in North Kivu; 80.8% (1,534/1,899) in Haut-Uélé; 75.3% (278/369) in Tshopo; and 73.5% (36/49) in South Ubangi.

1.2. Update on checkpoint and point of entry (PoE/PoC) activities

1.2.1. Main actions

Five (5) alerts were notified to the PoE/PoC in the last 24 hours, including three (3) in North Kivu, one (1) in Ituri, and one (1) in Haut-Uélé. In Ituri, a live alert intercepted at the Bey PoC (Komanda Health Zone) was validated and then transferred to the Komanda CT. In North Kivu, three (3) bodies were intercepted at the Mukulia (2) and Kasitu (1) PoCs in the Beni Health Zone; all were validated and sent to the morgue for swab. In Haut-Uélé, a (3) body intercepted at the PK10 PoC on the Poko road (Isiro Health Zone) was forcibly retrieved by the family before swab.

Table 4. Status of key PoE/PoC indicators by province, as of September 17, 2026

Indicators	Ituri	North Kivu	South Kivu	Boss	Haut-Uélé	Global
Report completeness	96,7% (58/60) 100%	(20/20) 55,6% (5/9)	100% (18/18) 85,7% (6/7) 93,9% (107/114)			
% of functional strategic PoE/PoC	100%	100%	100%	100%	100%	100%
Number of people who have passed PoE/PoC	154 818	99 988	6 882	18 068	11 801	291 557
% of travelers screened	97,0%	98,1%	98,2%	99,1%	99,0%	97,8%
% of travelers who washed their hands	97,0%	98,1%	98,2%	98,2%	99,0%	97,5%
% of travelers made aware	99,0%	98,1%	100,0%	100,0%	100,0%	99,0%
Number of alerts notified	1	3	0	0	1	5
Number of validated live alerts (cas suspects)	1	0	0	0	0	1
Number of bodies intercepted today % of suspected	0	3	0	0	1	4
cases transferred to CTE/CT	100% (1/1)	THAT	THAT	THAT	THAT	100% (1/1)
% of lifeless bodies swabbed/secured	THAT	100% (3/3)	THAT	THAT	0% (0/1)	75% (3/4)
% of suspected cases investigated within 24 h	100% (1/1)	THAT	THAT	THAT	THAT	100% (1/1)

1.2.2. Challenges

Suboptimal operationalization of PoE/PoC coupled with low reporting completeness in South Kivu (55.6%, 5/9); persistent insecurity hindering the commissioning of the Avakubi PoC (Ituri), low alert coding in DHIS2 Tracker, and insufficient mobility in Rwampara and Bunia. In North Kivu, lack of support from security personnel at the Butembo axis PoCs and threats against service providers. In South Kivu, absence of isolation measures at the Kavimvira, Kalundu Port SNCC, and Baraka PoEs.

1.3. Laboratory

1.3.1. Main actions

- **Ituri: 37 new positive results** (30 living and 7 deaths) out of 165 new samples received and analyzed (**positivity: 22.4%**).
- **North Kivu: 20 samples tested positive** (13 live and 7 dead) out of 171 samples analyzed (**positivity of 11.7%**).
- **Haut-Uélé: 4 new positive results** (3 living and 1 death) out of 21 samples received and analyzed at the Isiro laboratory (**positivity: 19.0%**).
- **Tshopo: 5 new positive results** (4 living and 1 death) out of 17 samples received and tested (**positivity: 29.4%**).

- **Bas-Uélé:** 3 samples received and analyzed (3 live) all tested negative.

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri,** 29 open rings were opened out of 41 expected (70.7%), and 152 households were decontaminated out of 154 identified (98.7%); 12 FOSA were decontaminated; 30 hygiene kits were provided in the Mongbwalu health zone and 78 kits were provided to schools.
- **In North Kivu,** 29 of the 44 planned decontamination rings were opened (65.9%), 11 of the 49 identified households were decontaminated (22.4%), and 23 of the 31 health facilities were decontaminated (74.2%). 47 scorecard assessments were conducted in 47 health establishments, and staff exposure risk assessments were carried out (104 of 198 providers were considered high-risk; 52.5%). Regarding the health surveillance system (HSS), 61 alerts were recorded, 58 HSS tests were carried out, and 60 bodies were swabbed (98.4%), with only one failure recorded in Katwa due to community resistance in the Vihole health district. Training for decontamination teams in the health zones of Kalunguta, Beni, and Mabalako was launched with support from IMA/MOMENTUM.
- **In Tshopo,** scorecard assessments were conducted at the Ndako ya Eliya Medical Center (30%) and the PNC Hospital (24%); training began for 95 primary healthcare workers in the Kabondo and Makiso-Kisangani health zones (WHO and IMC); and 14 health and social care (HSC) assessments were followed up in the 6 health zones, with 146 healthcare providers briefed on standard precautions. Regarding the health data collection (HDC), one alert was recorded and one body was taken to the Rekapi morgue, without any HDC being carried out.
- **In Haut-Uélé,** decontamination of the CTE (Treatment and Evaluation Center) and the internal medicine ward of the Boma Mangbetu General Referral Hospital, with briefings for patients and caregivers; decontamination of two public places in Wamba (territorial office and police station); and scorecard assessments of the ESS (Emergency Health Services) in Pawa and Libiki; a joint visit with the CREC (Regional Center for Emergency Response) to the Maranatha Church (Nsele Health Association, Isiro Health Zone). Regarding EDS (Emergency Health Services), 14 alerts were recorded and 4 EDS were carried out, with 2 refusals reported in the Tely Health Association (Isiro Health Zone).
- **In Bas-Uélé,** briefing and awareness-raising of 13 traditional healers and 6 service providers at control points on IPC, CREC and community-based surveillance; continued support for ESS in the health zones of Ango, Monga and Ganga.

1.4.2. Challenges

- **In North Kivu,** low coverage of household decontamination (22.4%; 11/49) and low opening of rings (65.9%; 29/44); decontamination teams in the health zones of Biena, Masereka, Kyondo, Mabalako and Vuhovi remain untrained; non-alignment of 86.6% of members of the IPC/WASH pillar on the motivation list; lack of mobility for the deployment of provincial IPC teams; urgent need for training for all IPC Data on data collection and management tools; insufficient handwashing facilities in schools.
- **In Tshopo,** insufficient IPC inputs in households, schools, markets and churches; low proportion of SSE with an IPC score above 80% (2.2%; 2/93); absolute resistance of cases 20 and 26 despite several negotiations to carry out public health actions; incinerator at HGR Makiso-Kisangani still out of order.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri,** there were 72 new admissions to the various dedicated facilities, compared to 84 discharges, including 12 recoveries. Thus, 430 patients are hospitalized out of 1015 beds, representing an occupancy rate of 42.4%; 362 out of 387 (93.5%) patients received hot meals; 10 out of 10 children are

under LNPE/SLM at the CTE, 4/4 at the nursery and 21/21 in the community; 33 patients are in critical condition and 3 transfers have been organised, including 1 from the Rwampara CTE to the Bunia HGR CTE and 2 from the Mangala CT to the Nizi CTE.

- **In North Kivu**, 122 new admissions were recorded compared to 95 discharges, including 6 recoveries; 403 Patients are hospitalized, including 217 in standardized facilities with a capacity of 228 beds, representing an occupancy rate of 95.2%, close to saturation; 73/73 patients, 4/4 accompanying persons and 52/52 frontline providers received a hot meal and 9/9 non-breastfed orphans are on breast milk substitute; 11 patients are in critical condition.
- **In Haut-Uélé**, there were 17 new admissions and 17 discharges, including 7 recoveries. Thus, 61 patients are hospitalized out of 138 beds, representing an occupancy rate of 44.2%; 86 accompanying persons and 34 PPLs received three hot meals, except in Rungu and Gombari; 7 ambulance trips were made for 7 patients transported.
- **In Bas-Uélé**, 6 new admissions reported; 11 patients remain in isolation including 1 confirmed case taken care of at the HGR of Ganga; 9 high risk contacts remain under prophylaxis and 10 have left unscathed from the follow-up.
- **In Tshopo**, 5 new admissions (3 PPL) were recorded with no discharges. Twelve (12) patients are in isolation out of 31 beds, representing an occupancy rate of 38.7%; 7 confirmed and 4 suspected cases received a hot meal and 42/42 (100%) frontline providers were served.
- **In South Kivu**, no new admissions have been recorded. Thirteen (13) patients are in isolation for 25 beds, i.e. an occupancy rate of 52.0%.

1.5.2. Challenges

Treatment remains partially substandard and capacity is limited: **In Ituri**, Ebola Treatment Centers (ETCs) are saturated with suspected cases in Nizi, Mambasa, Lolwa, Kilo, Mangala, and Mandima. **In North Kivu**, 46.2% of hospitalized patients are treated outside of standardized facilities; no beds are available in Butembo and Katwa. **In Tshopo**, the lack of safe blood and limited technical resources persist. **In Bas-Uélé**, the absence of ETCs and ambulances hinders the isolation of cases.

1.6. Vaccination

1.6.1. Main actions

- **In Tshopo**, 3,063 people have been vaccinated to date out of 11,703 targeted TPL/PPL (26.2%), including 1,684 in Makiso-Kisangani, 454 in Kabondo, 363 in Mangobo, 290 in Tshopo, 149 in Lubunga and 123 in Bafwasende; the remaining stock consists of 500 frozen doses and 937 thawed doses.
- **In Bas-Uélé**, 708 PPLs have been vaccinated to date, including 550 in Buta and 158 in Ganga; compassionate vaccination continues in Ganga with the 450 doses available and the transport of the ultra-cold chain to the PEV antenna in Buta continues (arrival of equipment in Titule).
- **In Ituri**, preparations for the vaccination of PPL continue.

1.6.2. Challenges

Low uptake of PPL and TPL in Tshopo, with a reported refusal in the Mandombe health district and the cancellation of the September 16, 2026 sessions in Lubunga, Mangobo, and Kabondo due to lack of mobility; the Bafwasende site remains underperforming (4% of the target population); vaccination rate at the Bafwasende site remains low (4% of the target population). (affected). In Bas-Uélé, there is no microplan for the vaccination of PPL in the six priority health zones (Aketi, Ango, Bili, Buta, Ganga and Viadana) and additional doses of ERVEBO are needed.

1.7. Continuity of Care

- **In Tshopo**, 146 cases received the experimental treatment, including 19 confirmed cases and 127 under post-exposure prophylaxis (92 completed, 11 interrupted, and 24 ongoing); CH 7 evaluation

Ndako ya Elikya (Makiso-Kisangani), validation of the Mpox drug distribution plan, reception of drugs from the UG-PDSS project and monitoring of PPLs; the temporary closure of the ESS DHK is being considered in the face of the increase in notified cases.

- **In Haut-Uélé**, deployment of medicines and supplies from the PDSS project of the Wamba BCZ to the health centers of Bolebole, Bakandja, and Mambati; evaluation of the Matali secondary hospital (loss of more than 90% of new antenatal care cases, 80% decrease in cesarean sections, 40% decline in follow-up of chronic patients, and safety stock depleted by 50%); and monitoring of 2 PPLs in the Pawa ZS
- **In Bas-Uélé**, end of post-exposure prophylactic treatment of PPL high-risk contacts in the Ganga health zone and awareness-raising of contacts for adherence to compassionate prophylaxis in Viadana.

1.8. Risk communication and community engagement

1.8.1. Main actions

- **In Ituri**, 14,515 home visits reached 42,689 people and 905 educational talks reached 10,737 people; 6 mass awareness campaigns reached 10,873 people, 21 community debates reached 467 people and 21 advocacy campaigns reached 38 people; 139 community alerts were notified (including 21 deaths), 36 of the 49 feedback issues were resolved within 48 hours and 248 community incidents were recorded (completeness: 8/28 health zones; 29%); support was provided for the pre-listing of 112 contacts around 11 confirmed cases, for the decontamination of 12/12 households, for 10 of the 15 EDS (67%) and for the transfer of 14 suspected cases.
- **In North Kivu**, 25,106 people were reached during 8,380 home visits, 1,797 during 69 educational talks and 2,336 during 34 mass communication actions; 272 community alerts notified out of 450 expected (60.4%), 7 feedbacks resolved out of 12 (58.3%), 6 of 9 incidents resolved (66.7%) and 1,134 people engaged out of 4,750 (23.9%); activation of a community brigade of 30 influencers in the Beni health zone, session with 41 pastors of the CBCA in Katwa and mass communication with 230 displaced people in Musimba (Musienene health zone).
- **In Haut-Uélé**, 4,009 worshippers were sensitized in Ibambi by the Provincial Governor; 3 educational talks reached 40 people, and 1 advocacy campaign reached 5 people; actions carried out around the 4 most recent confirmed cases reached 19 people (Pawa 8, Wamba 7, and Isiro 4). In Bas-Uélé, 24,504 people were reached in 6 of the 13 health zones, including 1,388 in schools (1,151 students and 237 teachers); 120 households were visited at home, 50 dialogues were held with community leaders, 16 alerts were raised by the Regional Community Health Teams (RECO), and 5 advocacy campaigns were conducted with Catholic-affiliated schools in Buta and with religious denominations.
- **In Tshopo**, 287 households visited at home reached 1,690 people (896 women and 794 men); preparatory communication to the EDS of the 30th confirmed case at the Rekapi morgue (Kabondo), awareness-raising in the CEVs of the Catholic Church and preparatory meeting for the training of shipowners and river supervisors (project "Congo River without Ebola"); 4 community alerts notified and all resolved.

1.8.2. Challenges

Persistent community resistance: in Tshopo, the confirmed case of escape from the CTE (20th cases) remain at home, refusals to list contacts and investigate suspects are reported to DHK around the 29th case and 5 resistances have been notified of which only 2 were broken.

In Ituri, data reporting was poor in some health zones (completeness: 8/28; 29%), there was a lack of quantified data on interventions around cases, and demotivation among stakeholders due to unpaid bills. In North Kivu, there was refusal to register cases and carry out decontamination in Butembo and Biena, threats against an infection prevention and control (IPC) worker, and denial of the disease. In Bas-Uélé, there was poor reporting of community alerts, demotivation among community response teams, and impassable roads.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- **In Ituri**, 19 people among the new confirmed cases and 89 of the 138 confirmed cases under follow-up (64.5%) benefited from SMSPS interventions, as well as 21 of the 54 new suspects (38.9%) and 98 of the 106 suspects under follow-up; 38 results announced, including 19 positive cases; 223 caregivers and 127 PPLs supported; 9/9 children followed up in daycare and 2 new admissions; 13 of the 17 households of separated children accompanied for 45 children followed up; 26 of the 31 recovered are followed up in the community and 6 of the 11 recovered are reintegrated; 8 of the 10 notified health zones are covered, for 10 of the 28 zones in the province; end of the capacity building session for 24 APS/TPS in 4 health zones.
- **In North Kivu**, 27 people among the new confirmed cases and 221 confirmed cases under follow-up were taken care of, as well as 217 new suspected cases and 276 suspected cases under follow-up; 113 results announced, including 27 positive cases, 77 PPLs supported, 78 households accompanied and 10/10 recovered and followed up in the community; 41/41 children followed up in the community, 3/3 in daycare and 29 of the 31 households of separated children accompanied (93.5%); 372 people (125 men, 178 women and 69 children) benefited from psychoeducation with the support of UNICEF and the 8 notified health zones are covered.
- **In Tshopo**, 1 of the last 2 confirmed cases (50%) and 2/2 confirmed cases under follow-up benefited from SMSPS interventions, as well as 4/4 new suspects and 1/1 suspect under follow-up; 6 results announced, including 2 positive ones, 3 households supported, 2 PPLs supported and 2 of the 5 resistances broken in the Kabondo health zone; 4 of the 7 notified health zones are covered (57.1%).
- **In Haut-Uélé**, 4 new confirmed cases and 20 confirmed cases under follow-up received SMSPS interventions, as well as 7 new suspected cases and 11 suspected cases under follow-up; 13 results were announced, including 8 positive cases; 46 caregivers and 43 people with disabilities received support; 5 separated children received community support; 15 household members received support; and 2 community health workers received assistance; 48 recovered individuals are still awaiting their unloading kits, and data from Pawa has not yet been transmitted. In Bas-Uélé, support for the confirmed case in Ganga remains limited by the lack of a psychologist.

1.9.2. Challenges

- **In Ituri**, low coverage of SMSPS interventions (10 of the 28 health zones covered). **In North Kivu**, Low coverage around the latest confirmed cases in nine health zones, lack of childcare facilities in 10 affected areas, and insufficient human resources. **In Tshopo**, there is no SMSPS (Social and Medical Prevention Service) provider in Wanierukula and Bafwasende. **In Haut-Uélé**, there are difficulties with the mobility of community health workers in Wamba and Boma Mangbetu, a lack of school and nutritional kits for separated children, and poor data reporting in Boma Mangbetu and Pawa. In Bas-Uélé, there is no psychologist in the Ganga health zone.

1.10. Logistics

1.10.1. Main actions

- Ensure the mobility of response teams in all affected provinces.

1.10.2. Security

1.10.3. Main actions

- **In North Kivu**, the pillar provided support and security coverage for IPC teams for the decontamination of households in the health districts of Mangote, Benegule, Kanzabi and Tuungane as well as the CH Sifa, CM La Brèche and CS La Charité facilities; the EDS teams were accompanied for the burial of confirmed cases at the family cemeteries of Mukulya and Bundji and for the swab of lifeless bodies at CH Maombi and Rugghenda, with 100% coverage of the interventions requested.

1.10.4. Challenges

In **North Kivu**, there is a lack of a permanent vehicle for the security team, a lack of financial incentives for agents and non-payment of police officers and security agents assigned to guard the ESS; lack of support from security elements in several PoCs along the Butembo axis (Kangote, Kyaghala, Mususa and Komba); delayed payment of actors involved in the response and non-compliance with commitments by some partners in several health zones.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions

- In **Ituri**, 40 participants including 21 women and 19 men were trained on PSEAH in the Bunia and Lita health zones for the benefit of healthcare personnel from the Lita CTE, APS and TPS from 10 health zones; 619 people whose target is ONEM agents benefited from awareness-raising on **PSEAH in the Bunia and Gethy health zones**.
- **Haut-Uélé**: 4 of the 6 health zones have carried out PSEA activities: 40 people sensitized in Isiro, 26 PRESICODESA in Gombari, 136 heads of schools in Wamba and 6 people in Pawa, 40 codes of good conduct having been signed; the 6 health zones know the reporting mechanism (100%).
- In **North Kivu**, a briefing was organized for 52 participants, including 23 men and 29 women. In the Katwa Health Zone, Vungi Health Area, this was followed by community consultations in 4 focus groups, one for men and youth, one for women, and one for people with disabilities.

1.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 13 of the 36 health zones in Ituri (36.1%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of some partners to align; insufficient mass awareness materials; lack of computer kits and office supplies for the pillar.

MESSAGES CLES

- In the provinces of Ituri, North Kivu and Haut-Uélé, transmission remains sustained, with a rebound in the province of Tshopo while in Bas Uélé, no confirmed cases have been reported in the last 48 hours.
- Thus, **66 new confirmed cases** were reported, including **16 community deaths**. In addition, **18 cases within treatment centers** were reported. The national total is **7,541 confirmed cases, including 3,639 deaths (case fatality rate: 48.3%)**.
- Ituri remains the epicenter; it accounts for **77.3 %** of confirmed cases and **56.1%** of new cases reported in the last 24 hours.
- The performance of epidemiological investigations remains satisfactory, with 100% of suspected cases investigated and a stable contact tracing rate of **87.2%**.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains: Rapidly increase reception capacities in CTE/CT in North Kivu.
- Strengthen preparedness and response to Ebola Virus Disease-B in the provinces of the River Corridor and surrounding areas.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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